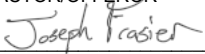
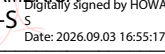


AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT				1. CONTRACT ID CODE		PAGE OF PAGES 1 45	
2. AMENDMENT/MODIFICATION NO. 00002		3. EFFECTIVE DATE 09/03/2026		4. REQUISITION/PURCHASE REQ. NO. 000HCRDB-2026-00146		5. PROJECT NO. (If applicable)	
6. ISSUED BY OFR - Office of Acquisition Services Chamblee Campus Building 108 4770 Buford Highway Atlanta, GA 30341-		CODE 15948		7. ADMINISTERED BY (If other than Item 6)		CODE	
8. NAME AND ADDRESS OF CONTRACTOR (No., street, county, State and ZIP Code) Frasier Digital, LLC 20915 Mystic Stone Dr Tomball, Harris County, TX 77375				(√)		9A. AMENDMENT OF SOLICITATION NO. RFQ 75D301-26-Q-00146	
				X		9B. DATED (See Item 11)	
						10A. MODIFICATION OF CONTRACT/ORDER NO.	
						10B. DATED (See Item 13)	
CODE		FACILITY CODE					
11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS							
☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods: (a) By completing Items 8 and 15, and returning ____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEDGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.							
12. ACCOUNTING AND APPROPRIATION DATA (If required)							
13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.							
(√)		A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: (Specify authority) THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.					
		B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES (such as changes in paying office, appropriation date, etc.) SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).					
		C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:					
		D. OTHER (Specify type of modification and authority)					
E. IMPORTANT: Contractor ☐ is not, ☐ is required to sign this document and return ____ copies to the issuing office.							
14. DESCRIPTION OF AMENDMENT/MODIFICATION (Organized by UCF section headings, including solicitation/contract subject matter where feasible.)							
This solicitation amendment is issued to update the PWS to include English and Spanish options as indicated in Question #270. This amendment also provides an answer to Question #116.							
Howard D Bish Contracting Officer uwo8@cdc.gov							
Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.							
15A. NAME AND TITLE OF SIGNER (Type or print) Joseph Frasier, Founder and Managing Member Frasier Digital, LLC				16A. NAME OF CONTRACTING OFFICER Howard D Bish			
15B. CONTRACTOR/OFFEROR  (Signature of person authorized to sign)		15C. DATE SIGNED 09/04/2026		16B. UNITED STATES OF AMERICA HOWARD D. BISH -S BY  (Signature of Contracting Officer)		16C. DATE SIGNED 9/3/2026	

RFQ 75D301-26-Q-00146

Modernized Application for Consumer and Healthcare Provider VAERS Reporting
Questions and Answers

1. Intelligent completion assistance (PWS Section 2; Task 1). Does the Government envision the "intelligent validation that assists with form completion, answers user questions, and supports site navigation" as a conversational/AI-assistant component, or as rules-based intelligent form validation and contextual guidance? If AI-based approaches are acceptable, may the application leverage CDC's existing FedRAMP-authorized Azure OpenAI services (e.g., via the EDAV environment) rather than introducing new AI services?

Rule based form validation and AI for contextual guidance. Yes on EDAV or other FedRAMP authorized provider.

2. Relationship to existing VAERS contracts (PWS Section 3). The PWS directs coordination with "other existing contracts supporting the VAERS system." Please confirm this requirement is a new capability with no incumbent contractor performing this scope, and briefly describe the division of responsibilities between this contract and the existing VAERS support contracts.

This is a new capability with no incumbent contractor performing this scope of work.

Contract: #75D30124F19979

Contractor: General Dynamics Information Technology, Inc (GDIT)

Provides technical and programmatic support services to collect and analyze information on vaccine adverse events (VAEs) and facilitate reporting to VAERS.

Contract: # 75D30126C20929

Contractor: EEC-Lukos JV, LLC

Provides CDC with a ready source of clinical, scientific, and technical services for the review, abstraction and revision/updating of Medical Dictionary for Regulatory Activities (MedDRA) coding of VAERS reports and associated medical records.

3. Page-limit treatment of required plans (Section F; CDCL.03/CDCL.10). Please confirm that the Data Management Plan and the AI Compliance and Risk Management Plan are excluded from the 15-page Technical Proposal limitation, and identify where the CDCH.10 template referenced in CDCL.03 may be obtained.

The 15-page limitation applies to the Technical Proposal. All other items may be submitted as attachments and are not counted towards the 15-page limit. Resumes can be submitted as an attachment and do not count against page limit.

4. Prototype (Tab 2-2). Please confirm that the prototype may be hosted on offeror-provided infrastructure using synthetic data only, and state how long after the submission deadline the prototype link must remain accessible for evaluation.

Confirmed. The prototype may be hosted on offeror-controlled infrastructure using synthetic/non-production data only. The link must remain live and unchanged through the conclusion of

evaluation; offerors will be notified if earlier release is appropriate.

5. Paperwork Reduction Act. The PWS notes the branching-logic form and new survey instruments may trigger PRA review. Please confirm that PRA clearance activities are a Government responsibility and describe how PRA timelines are expected to interact with the DOA+90-day beta and DOA+150-day production milestones.

Yes, it's a government responsibility and we will work with the awardee to address timeline changes where appropriate.

6. Question: Section F establishes a 15-page limit for the Technical Proposal and excludes cover pages, tables of contents, and resumes. The solicitation separately requires a Data Management Plan (per CDCL.10, evaluated under CDCM.03) and an Artificial Intelligence Compliance and Risk Management Plan (per CDCH.10 and CDCL.03, evaluated under CDCM.04), but neither appears in the Technical Volume tab structure in Section F. Please confirm whether the Data Management Plan and the AI Compliance and Risk Management Plan (a) count against the 15-page Technical Proposal limit, or (b) should be submitted as separate attachments outside the page limit.

See answer to Question 3.

7. Will the Government consider offerors whose relevant experience consists primarily of commercial software modernization and custom application development projects, provided they can clearly demonstrate the technical capability to perform the work? Or is prior experience performing similar work specifically for the CDC or another Federal agency expected to be a significant evaluation factor?

Yes. Relevant commercial or non-federal experience demonstrating comparable technical complexity, healthcare workflows, and regulatory/PHI-PHI requirements will be considered under the Similar Experience factor.

8. Additionally, is the Government open to proposals from newly established small businesses that demonstrate strong technical capability and a credible execution approach, even if they have limited federal prime contract past performance?

Yes. The Government will evaluate demonstrated technical capability and credible execution approach; extensive federal prime experience is not a mandatory prerequisite.

9. Scope of security authorization: ATD versus ATO.
PWS Section 5 and the stated objective describe the contractor producing security and privacy documentation sufficient for CDC, in coordination with OCIO, to issue an Authority to Develop. Section 11 also references an Authority to Operate, and the Standard 3 and Standard 4 requirements describe a full Security Assessment and Authorization package due within 120 days of the planned go-live date, including SSP, SAR, POA&M, contingency plan and test, E-Authentication assessment, and penetration test results.

A development environment for the new VAERS user interface can be established within CDC's OCIO-hosted infrastructure using authorized software under a Change Request (CR). Once all requirements are documented, a change request can be completed within weeks. Those requirements include items such as identification and documentation of

software, architecture, configurations of products, vulnerability scans (once stood up), firewall rules, etc. package. Government will provide existing information, as applicable. Awardee is responsible for updating the security and privacy documentation package.

10. Please confirm the contractor's responsibility. Specifically, is the contractor deploying into CDC's existing FedRAMP-authorized Azure environment and producing documentation to support CDC's issuance of the ATD and ATO, or is the contractor responsible for obtaining a separate system authorization, including any third-party assessment organization involvement?

[See answer to Question 9.](#)

11. Placement and page treatment of required standalone plans.
Section L identifies the Volume I structure as Tab 1 Executive Summary, Tab 2 Technical Approach, Tab 3 Management Approach, and Tab 4 Similar Experience, subject to the 15-page limitation. Separately, the solicitation requires a Data Management Plan, an Artificial Intelligence Compliance and Risk Management Plan, an Incident Response Plan, and a Section 508 Accessibility Conformance Report.

[See answer to Question 3.](#)

12. Please confirm where these documents should be placed in the quotation, and whether they are excluded from the 15-page limitation in the same manner as cover pages, the table of contents, and Key Personnel resumes.

[See answer to Question 3.](#)

13. Final delivery date. The deliverables schedule lists the Testing and Final Release (Production-Ready) deliverable as due both "DOA + 150 days" and "by end of period of performance." Please confirm which date governs.

[No separate option CLIN exists in Section B. Offerors are advised to rely on the solicitation as written; no separately priced option period is required. DOA + 150 days governs.](#)

14. Option to extend services.
FAR 52.217-8, Option to Extend Services, is incorporated, and FAR 52.217-5 provides that option prices are added to the total for evaluation. The schedule does not include a priced option line item. Please confirm whether offerors should price an option period, and if so, in what format.

[FAR 52.217-8 is removed from the solicitation and will not be included any resulting award. This requirement is considered to be for nonseverable services which means that it is a specific entire job or undertaking, resulting in a defined end product, and provides value to the Government only at the time the entire job is completed.](#)

15. Vulnerability remediation timeframes.
The POA&M requirements state remediation timeframes of 15 days for critical and 30 days for high findings, with medium findings shown as 60 days in one section and 90 days in the vulnerability management and cloud sections. Please confirm the governing timeframe for medium findings.

It is recommended to use 60 days for any planning purposes.

16. Submission email address.

The solicitation shows the submission address as uwo8@cdc.gov in most places, and as uw08@cdc.gov in one instance in the instructions. These differ by a letter "o" versus a zero. We are submitting to uwo8@cdc.gov and wanted to flag the discrepancy in case it is a typographical error that could affect other offerors.

Submissions should be made to: uwo8@cdc.gov.

17. Evaluation factors. Will the Government confirm the evaluation factors and their relative importance for this quotation, including whether technical approach and past performance are evaluated factors?

The evaluation factors are as indicated in solicitation Section E:

Factor 1: Technical

Sub-Factor 1: Technical Approach

Sub-Factor 2: Management Approach

Sub-Factor 3: Similar Experience

Factor 2: Price

Factor 3: Past Performance

Evaluation factors are listed in descending order of importance. Evaluation factors other than price when combined are significantly more important than price.

18. Past performance sources. Given the small-business set-aside, will the Government consider recent commercial or non-federal past performance, and the past performance of proposed subcontractors or teaming partners, in the evaluation?

Yes.

19. Quote format. Are there page limits, formatting requirements, or specified volumes/sections for the technical and price quote?

See solicitation Section F – Instructions to Offerors.

20. Teaming and subcontracting. Does the Government anticipate that offerors may team or subcontract to deliver the full scope across development, UX/Section 508, and security authorization, and are there limitations on subcontracting beyond the standard clause?

Teaming and subcontracting are both permissible approaches. There are no limitations on subcontracting beyond the standard clauses.

21. Security authorization. Will the Government clarify the expected division of responsibility for the SA&A/ATO package between the contractor and CDC/OCIO, and whether an existing System Security Plan or defined authorization boundary will be provided as a starting point?

See answer to Question 9.

22. Incumbent and access timing. Is there an incumbent currently supporting VAERS reporting, and if so, will incumbent-developed code, designs, or artifacts be furnished as Government-Furnished

Information at kickoff? Relatedly, given the Tier 2 background investigation requirement and the nine-month period of performance, when does CDC anticipate issuing PIV credentials and granting Azure environment access relative to award?

Offerors are advised to rely on the solicitation as written. No further information regarding incumbent status is provided. PIV credentials and Cloud environment access timing relative to award will be addressed at kickoff. See also Question 13 regarding schedule.

23. Section 508 deliverables. Will the Government confirm whether the required ACR/VPAT applies to the final application only or also to interim/beta releases, and whether manual assistive-technology testing is expected in addition to automated scanning?

The ACR/VPAT is due with Beta Release (draft) and updated with Final Release. Manual assistive-technology testing is expected in addition to automated scanning.

24. We noticed the solicitation has 3 modifications recorded on [SAM.gov](https://sam.gov). Are there any updated or additional documents we should review beyond the RFQ PDF currently attached?

No – the only prior modification was to add the following update:

8/18/2026 Updates: 1. Questions are due 8/20/2026 2:00 PM ET. 2. Add Attachment 1 Artificial Intelligence (AI) Use Compliance and Risk Management Plan.

25. Could you confirm the preferred submission format — are quotes to be submitted via email to this address, and are there any page limits or specific formatting requirements?

See answers to questions 3 and 16.

26. For the Similar Experience Factor, may the Government consider relevant experience performed by proposed subcontractors and key personnel, or must the experience have been performed under a contract held directly by the prime Offeror?

Subcontractor and key personnel experiences may be considered under the Similar Experience factor.

27. If subcontractor experience may be considered, are there any minimum workshare, commitment-letter, or documentation requirements beyond the draft subcontracting agreement identified in the RFQ?

No, only a written draft Subcontracting Agreement is required. No additional minimum workshare documentation is required.

28. For the proposal prototype, would an interactive prototype using synthetic data and demonstrating representative workflows, branching logic, validation, accessibility, and user assistance satisfy the requirement, or is the Government expecting a functioning full-stack application connected to a database?

Yes, prototypes may be on Contractor controlled infrastructure, and no restrictions as long as the prototype tech stack uses the same tech stack available and operated at CDC and are part of FedRAMP approved and operated infrastructure.

29. Must the proposal prototype be hosted within a FedRAMP-authorized environment, or may it be hosted in a commercially available environment provided that it contains no VAERS data, PHI, PII, or other Government information?

See answer to question 28.

30. Does the Government require the “intelligent validation,” form-completion assistance, supplemental-document suggestion tool, and interactive FAQ capabilities to use artificial intelligence, or may Offerors propose deterministic rules-based functionality where appropriate?

Offerors may propose deterministic rules-based functionality where appropriate; AI is not mandated, but considered helpful for contextual follow up question assistance.

31. If an Offeror proposes no AI use in its solution, is an Artificial Intelligence Compliance and Risk Management Plan still required with the proposal? If a plan is required, should the Offeror submit the provided attachment indicating that AI is not proposed?

Yes, an AI Compliance and Risk Management Plan is required with every proposal. If no AI use is proposed, submit the attachment indicating no AI use.

32. Will the Government provide the selected contractor with the current VAERS data dictionary, interface specifications, validation rules, integration documentation, representative test scenarios, and baseline performance data following award?

Yes, these will be furnished at kickoff per PWS Section 9.

33. The RFQ requires resumes for all proposed key personnel, while the Key Personnel table in Section D does not identify specific positions. May Offerors determine and propose their own key personnel positions based on their technical and management approaches?

Yes, offerors may propose their own Key Personnel positions consistent with their technical and management approach. The Government did not identify any specific positions as key personnel positions.

34. Please confirm whether the 15-page Technical Proposal limitation includes the prototype documentation and the Artificial Intelligence Compliance and Risk Management Plan, and whether the Data Management Plan is included in or excluded from that limitation.

See answer to Question 3.

35. Will the Government accept relevant commercial healthcare application experience as Similar Experience when it demonstrates comparable healthcare workflows, PHI/PII safeguards, regulatory requirements, technical complexity, and stakeholder coordination, even if the work was not performed for a federal customer?

Yes, provided it demonstrates comparable workflows, PHI/PII safeguards, regulatory requirements, complexity, and stakeholder coordination.

36. Page limit. Section II states that cover pages, tables of contents, and resumes are excluded from the fifteen-page Technical Proposal limit. HHSAR 352.239-78 separately requires an Accessibility Conformance Report, and CDCL.03 requires an AI Compliance and Risk Management Plan where AI use is identified. Do those two documents count against the fifteen pages, or are they submitted as separate attachments alongside Volume I?

See answer to Question 3.

37. Signature. Block 14 of the SF-18 calls for the signature of the person authorized to sign the quotation. Does CDC accept a typed or electronic signature there, or does the Government require a scanned wet signature?

An electronic signature is acceptable.

38. Page limitation and required plans. Section F states the Technical Proposal shall not exceed 15 pages, with only cover pages, tables of contents, and resumes excluded. The solicitation also requires several plans and documents at proposal time — the AI Use Compliance and Risk Management Plan (Attachment 1 template, CDCH.10/CDCL.03), the Data Management Plan (CDCL.10), the Incident Response Plan (listed in the deliverables table as due "as part of Proposal"), and the Section 508 Accessibility Conformance Checklist or ACR (HHSAR 352.239-78(b)/(d)). Do these documents count against the 15-page Technical Proposal limit, or may they be submitted as separate attachments excluded from the page count?

See answer to Question 3.

39. Section 508 submission at proposal time. HHSAR 352.239-78(b) and (d) require an HHS Section 508 Accessibility Conformance Checklist or an ACR/VPAT with the proposal. Because this acquisition is for the development of an application that does not yet exist, would the Government please confirm which submission it expects: (a) an HHS Section 508 Accessibility Conformance Checklist completed for the ICT services to be developed, (b) an ACR/VPAT prepared against the offeror's prototype, or (c) both? If a specific checklist from the HHS library is preferred, please identify it.

See answer to Question 23.

40. AI Use Compliance and Risk Management Plan format. Should the completed Attachment 1 template be submitted as its own document (Word or PDF attachment), or should its content be reproduced within Volume I? If submitted as its own document, please confirm it is not counted against the 15-page limit (see Question 1).

See answer to Question 3.

41. Prototype accessibility from the CDC network. The prototype must be provided as an accessible electronic link that the Government can reach and evaluate. Will the evaluation team be able to access an externally hosted, publicly reachable HTTPS website on a non-.gov commercial domain from the CDC network, and are there any restrictions on hosting location, domain, or site technology the Government would like offerors to observe? Also, through what date should offerors keep the prototype link live and unchanged?

Yes, evaluation team can access the link to an externally hosted, publicly reachable commercial domain. However, the prototype tech stack used should be the same tech stack available and operated at CDC and are part of FedRAMP approved and operated infrastructure (see also answer to question 28).

The link should remain available throughout the evaluation process.

42. Timing of Government-furnished access. PWS Section 5 states that PIV cards and laptops will be furnished by the Government and that CDC will provide access to the CDC-managed Azure environment. Given that the staff roster is due 30 days before the contract effective date and that background investigations and training precede system access, when — relative to the 09/28/2026 effective date — should offerors assume contractor personnel will receive

PIV credentials, Government laptops, and CDC Azure environment access? Should proposed schedules assume development environment access is available on the effective date, or at some later milestone?

Offerors should assume development environment access may not be available on / within two weeks of start date and should build reasonable ramp-up time into their schedules; milestone dates will be adjusted for Government-caused delays consistent with standard FAR delay provisions.

43. Paperwork Reduction Act clearance. The PWS states the Government will determine PRA applicability and recommends early CO/PRA engagement, and HHSAR 352.211-3 requires the contractor to allow at least 120 days for OMB clearance and prohibits data collection before written CO authorization. Within the nine-month period of performance, does CDC hold existing OMB clearance covering the redesigned reporting form and the new satisfaction survey instruments (PWS 1.5 and 1.7), or should offerors plan and price contractor support for a new or revised information collection request? What schedule assumption would the Government like offerors to use for PRA clearance?

See answer to question 5.

44. Baseline for the abandonment-rate requirement. PWS 1.10 and PRS require reducing the submission abandonment rate by at least 30 percent compared to the baseline established at project kickoff. Will the Government provide current VAERS submission analytics — abandonment rate, completion time, and related metrics — for establishing that baseline, or is the contractor responsible for instrumenting the current system and measuring the baseline at kickoff? If the latter, is measurement of the existing system within the contract scope?

The Contractor is responsible for instrumenting the existing system and establishing the baseline after project kickoff; this is within scope. The Government does not currently maintain this analytics baseline.

45. Existing application and upload tool documentation. PWS 2.1 requires integrating a new medical-record upload capability while maintaining the current post-submission upload tool, and PRS#9 evaluates preservation of that tool. Will documentation, interface specifications, or source code for the current VAERS reporting application and the existing post-submission upload tool be made available to the awardee as Government-furnished information, and at what point after award?

Available documentation will be furnished at kickoff; existing source code will be made available to the extent it exists and is releasable.

46. Option pricing. The schedule contains CLINs 0001 through 0003 with no separately identified option period, while FAR 52.217-8 (Option to Extend Services) is incorporated and the evaluation provision states that option prices will be added to the base for award purposes. Should offerors include separately priced amounts for the potential six-month extension of services, and if so, under what CLIN structure should those amounts be presented in the Price Proposal?

CLIN 0001 is the primary CLIN for this project. CLINs 0002 and 0003 are travel CLINs that the Government will fund if determined to be necessary. FAR 52.217-8 is removed from the solicitation. Also, see answers to questions 13 and 14.

47. Set-aside status. Block 10 of the SF-18 is difficult to read in the posted PDF, and the notice on SAM.gov does not display a set-aside type. Could you confirm whether this RFQ is a small business set-aside?

[This requirement is a 100% small business set-aside.](#)

48. Contract support hours. PWS Section 6, Subsection B requires contract support during CDC core business hours, 8:30 a.m. to 5:00 p.m. ET. Could you clarify what that requires in practice: a continuously staffed availability window, or availability for scheduled coordination and Government-initiated contact within a defined response time? If there is an expected response time we would like to price to it rather than guess.

[Availability for scheduled coordination and Government-initiated contact within a reasonable response time is sufficient. Offerors should propose their own reasonable response-time commitment.](#)

49. SA&A package. The RFQ requires the contractor to complete and maintain an agency SA&A package to support the agency ATO. As VAERS is an existing system, does CDC provide existing artifacts such as a current System Security Plan, contingency plan or prior POA&M as a starting point, or is the package expected to be developed from scratch?

[Government will provide existing system artifacts and information, as applicable. You are not expected to create a package from scratch.](#)

50. Proposal Prototype Hosting and Environment
Section F requires offerors to provide an accessible electronic link to the prototype for evaluation. Please confirm that the proposal-stage prototype may be hosted in a contractor-controlled commercial environment outside the CDC Azure environment, using synthetic/non-production data, and that CDC security authorization/ATO requirements apply to the post-award application rather than the proposal prototype.

[See answer to questions 28 and 41.](#)

51. Draft Subcontracting Agreement and 15-Page Limit
Section F limits Volume I, Technical Proposal, to 15 pages, excluding cover pages, tables of contents, and resumes. Tab 3-1 also requires offerors proposing subcontractors to provide a written copy of the proposed draft Subcontracting Agreement. Please confirm that the draft Subcontracting Agreement may be submitted as a separate attachment and is excluded from the 15-page Technical Proposal limitation.

[See answer to Question 3.](#)

52. Data Management Plan and 15-Page Limit
CDCL.10 requires a Data Management Plan as part of the proposal and states that a proposal received without a DMP will be deemed unacceptable. Please identify the proposal volume or attachment in which the DMP should be submitted and confirm whether it is excluded from the 15-page Technical Proposal limitation.

[See answer to Question 3.](#)

53. FedRAMP / ATO Authorization Boundary
The PWS states that the application will be developed and deployed within the CDC-managed FedRAMP-authorized Azure environment and that the contractor will support CDC/OCIO

issuance of the ATD and ATO. Standard-4 also references a FedRAMP-compliant ATO, 3PAO assessment, and agency SA&A. Please clarify whether the contractor-developed application operating entirely within CDC's managed Azure environment is expected to obtain a separate contractor/vendor FedRAMP authorization and 3PAO assessment, or whether the contractor is expected to support CDC's application-level ATD/SA&A/ATO within the existing CDC cloud authorization boundary.

No, the contractor will not need to obtain a separate ATO. The existing VAERS Azure environment operates under CDC's existing authorization. The new application will be evaluated for inheritance of applicable controls where appropriate, subject to CDC/OCIO's determination during the ATD/ATO process. The Contractor is responsible for updating the security and privacy documentation package. See answer to Question 9.

54. Final Release Due Date

Section 8 identifies the Testing and Final Release (Production-Ready) deliverable as due "By end of PoP (DOA + 150 days)." The stated Period of Performance is September 28, 2026 through June 27, 2027, which is longer than 150 days. Please clarify whether the Production-Ready Final Release is due 150 days after date of award or at the end of the nine-month Period of Performance.

See answer to Question 13.

55. Day-45 Alpha Integration Expectation

Section 8 requires a Working Prototype (Alpha) Demonstration Build within 45 calendar days, while Section 9 states that VAERS data-element definitions, business rules, and integration requirements will be provided at kickoff. Please clarify whether the Day-45 Alpha is required to be integrated end-to-end with the existing VAERS environment, or whether the Alpha may demonstrate the production application workflow and VAERS-compatible structured output using a staged or simulated integration boundary, with operational VAERS integration completed during subsequent Beta/Final activities.

The Alpha may demonstrate the production workflow and VAERS-compatible structured output using a staged or simulated integration boundary; full operational integration is completed during subsequent Beta/Final activities.

56. Section 508 Documentation at Proposal Stage

HHSAR 352.239-78 on page 34 requires offerors to submit an appropriate HHS Section 508 Accessibility Conformance Checklist or Accessibility Conformance Report (ACR/VPAT) with their offer. This solicitation is for development of a new custom web application that will not exist in completed form at the time of proposal submission, and Section 8 separately requires an ACR/VPAT with the Beta Release, updated with Final. Please clarify what Section 508 documentation the Government requires at proposal submission for a not-yet-developed custom application, and whether an applicable HHS accessibility checklist addressing the proposed development approach is sufficient at this stage.

See answer to Question 23.

57. Magnitude. Can CDC share the IGCE range or the estimated level of effort (labor hours or FTEs) for CLIN 0001? Is this acquisition being conducted under the simplified acquisition threshold?

The CDC will not disclose the Independent Government Cost Estimate (IGCE) nor an estimated level of effort. The CDC is intentionally allowing the contractor community to independently

assess the requirements, leverage its industry expertise, and propose the most effective, efficient, and innovative solution to meet the Government's objectives.

58. Hosting and authorization. The PWS places the application inside CDC's FedRAMP-authorized Azure environment (Task 3.2, Section 11). Please confirm the contractor is not expected to hold or obtain its own FedRAMP cloud service provider authorization, and that the SA&A obligation is the agency ATD/ATO package for the application within CDC's existing boundary.

See answer to Question 9.

59. Prototype (Tab 2-2). May the prototype be hosted on the offeror's own infrastructure using synthetic data, and does it need to remain available only at submission or through the evaluation period?

See answer to questions 28 and 41.

60. Incumbency. Is there a current contractor supporting the VAERS public reporting front end, and is this scope new work or a re-compete?

See answer to question 2.

61. Key personnel and experience. How many key personnel positions does CDC expect? Can the healthcare informatics experience evaluated under Tabs 3-2 and 4 be satisfied by proposed key personnel or a subcontractor rather than the prime's corporate history, consistent with FAR 15.305(a)(2)(iii)?

The determination to identify key personnel is at the offeror's discretion. Yes, key personnel or subcontractor experience can satisfy health informatics experience rather than prime contractor's corporate history.

62. 1CDP. Is integration with the One CDC Data Platform (Palantir Foundry) in base scope, or is delivery of VAERS-compatible structured data mappings under Task 1.9 sufficient?

Delivery of VAERS-compatible structured data mappings under Task 1.9 is sufficient; 1CDP integration is not in base scope.

63. Contract vehicle. Task 4 and the Management Approach instructions refer to a "task order" and an "incumbent Contractor." Please confirm this is a standalone firm-fixed-price contract and not an order under an IDIQ or BPA.

This is a stand alone firm-fixed-price contract. All references to "task order: in the solicitation should be to "contract." There is no incumbent. This requirement is not an order under an IDIQ or BPA.

64. Background investigations. Section 11, paragraph 5 (Position Sensitivity Designations) is blank. What investigation tier applies to contractor staff?

Public Trust Tier 2 applies.

65. Section F.I excludes only cover pages, tables of contents, and resumes from the Volume I 15-page limitation. The solicitation separately requires a Data Management Plan (CDCL.10), an AI Compliance and Risk Management Plan where AI use is planned or potential (CDCL.03/CDCH.10), a Section 508 Accessibility Conformance Checklist or ACR/VPAT

(HHSAR 352.239-78), and an Incident Response Plan “as part of Proposal” (PWS Standard-4). Please confirm if these solicitation-required plans and artifacts (a) are excluded from the Volume I 15-page limitation, and if not, (b) where they should be submitted.

See answer to Question 3.

66. If subcontractors are proposed, may the required written copy of the draft Subcontracting Agreement be submitted as an appendix, excluded from the Volume I 15-page limitation?

See answer to Question 3.

67. Section B pre-prints CLINs 0002 and 0003 at \$31,500.00 each (“Estimate” / “Not-to-Exceed”). Should quoters carry these pre-printed NTE amounts as the quoted unit and extended prices for CLINs 0002–0003 (with itemized travel estimates provided as supporting back-up documentation), or should quoters propose actual estimated travel amounts below the NTE? Please also confirm how CLINs 0002–0003 will be treated in the total evaluated price.

Offerors are to all use the same unit and extended NTE prices for CLINs 0002 and 0003. Both CLINs should have a unit price and NTE extended price of \$31,500.00.

68. FAR 52.217-8 is included in the contract clauses. Will the potential six-month extension of services be included in the total evaluated price (e.g., by adding up to six months of performance at contract rates), and if so, how should quoters present pricing for the extension period?

See answer to question 14.

69. Please confirm which provision fill-ins, representations, and certifications must be completed and returned with the quotation (e.g., FAR 52.203-11, 52.203-18, 52.204-7, 52.229-11, and 52.240-90), whether current SAM.gov representations and certifications satisfy any of these requirements, and whether the CDCL.09 CPARS contractor representative information is required with the quotation or at award.

Offerors are to complete any fill-ins, representations, and certifications required by the FAR provisions indicated in the solicitation. If they are part of the current SAM.gov representations and certifications then separate submissions are not required. Offerors are to complete CDCL.09. The documentation can be submitted as part of the Price Proposal.

70. Section F.I directs quotations to uw08@cdc.gov but directs questions “exclusively by email to uw08@cdc.gov,” while page 1 directs questions, quotes, and all communication to uw08@cdc.gov. Please confirm the correct e-mail address(es) for (a) questions and (b) quotation submission, and whether the Government will acknowledge receipt of quotations.

See answer to Question 16.

71. CDCL.03 requires the AI Compliance and Risk Management Plan to be submitted “using the template provided,” but no template accompanies the RFQ document, and the invoicing instructions reference a “Section J, List of Attachments” that the RFQ does not contain. Please provide the AI plan template (Attachment 1) or identify where it is posted, and confirm the complete list of attachments to this solicitation.

The AI plan attachment was added to the solicitation notice on 8/18/2026.

8/18/2026 Updates: 1. Questions are due 8/20/2026 2:00 PM ET. 2. Add Attachment 1 Artificial Intelligence (AI) Use Compliance and Risk Management Plan.

72. For the prototype submitted through an accessible electronic link: are credential-protected links acceptable if access credentials are provided in the proposal? Should the link remain accessible and unchanged through the full evaluation period, and will evaluators access it from the CDC network (relevant to any firewall or allow-listing constraints)?

See answer to questions 28 and 41. The link should remain through evaluation period and accessors will access it from the CDC network.

73. The RFQ does not state where the completed and signed SF 18 should be placed within the two-volume submission. Please advise where the Government prefers the signed SF 18 to be placed. Please also confirm that quoters may annotate the SF 18 schedule block with “See Price Breakdown Worksheet” in lieu of restating pricing on the form.

The signed SF 18 is to be included with the Price Proposal. Offerors may annotate the SF 18 schedule with “See Price Breakdown Worksheet” in lieu of restating pricing on the form.

74. Correct email address for proposals and for questions

Section F, page 61, directs proposals to uwo8@cdc.gov and questions to uw08@cdc.gov. Please confirm which address is correct for each, so that we can be certain our proposal volumes reach the Contracting Officer. We have copied both addresses on this message for that reason.

See answer to Question 16.

75. Attachment 1 and the 15-page Technical Proposal limitation

Amendment 0001 adds Attachment 1, the Artificial Intelligence (AI) Use Compliance and Risk Management Plan. Section F excludes cover pages, tables of contents and resumes from the 15-page limitation by name, and is silent on attachments. Please confirm whether Attachment 1 counts toward the 15-page Technical Proposal limitation, or is excluded from it.

We would ask the same of the Data Management Plan required by CDCL.10 and the Accessibility Conformance Report required by HHSAR 352.239-78, both of which are submitted with the proposal and neither of which is named in the Section F tab structure.

See answer to question 3.

76. HHS onboarding and the period of performance

The PWS requires a completed background investigation through HHS onboarding for all contractor and subcontractor personnel before work begins, with a period of performance starting 28 September 2026. Please confirm whether the onboarding period falls within the period of performance, and advise the Government's expected processing time, so that offerors can schedule accordingly.

See answer to question 42.

77. Government-furnished information on the existing application

Please confirm what the Government will furnish at award regarding the existing VAERS reporting application, specifically source code, data model documentation, design assets, and any current Accessibility Conformance Report, and whether access to the CDC-managed Azure environment is provided at start of performance.

Available documentation, including any existing ACR, will be furnished at kickoff to the extent it exists and is releasable. Access to the CDC-managed Azure environment is provided per the ATD-governed schedule discussed in Question 22.

78. Is the working prototype a scored evaluation factor, or a supporting exhibit to the Technical Plan?

The prototype (Tab 2-2) is a scored element of the Technical Proposal evaluation.

79. Task 1.1 requires requirements gathering after award. On what requirements does CDC expect offerors to base a pre-award prototype?

Offerors should base the prototype on the publicly available current VAERS form and general PWS requirements. Full data elements and business rules will be provided at kickoff. The current form can be found at [VAERS - Download / Upload a Writable PDF Form](#) and the VAERS Data Use Guide found at [VAERS - Data Sets](#) provides field names and form box alignment (derived fields are related to this RFQ).

80. What minimum functionality must the prototype demonstrate?

See answer to Question 28.

81. Will CDC release the current VAERS form data elements and business rules before the due date so prototypes can be based on them? Section 9 currently provides them at kickoff.

Offerors should base the prototype on the publicly available current VAERS form and general PWS requirements. Any additional data elements and business rules will be provided at kickoff. See question 264 for specific public artifacts available.

82. If there are unforeseen delays from the Government side for ATD issuance, PIV provisioning, or EPLC stage gate review, will the Day 45 Alpha, Day 90 Beta, and final release milestones be adjusted?

Yes, milestone dates will be adjusted to the extent delays are attributable to Government dependencies, consistent with standard FAR provisions.

83. Section 5 states CDC will provide access to the CDC-managed Azure environment for development, testing, and deployment, governed by the ATD. Please clarify whether the Contractor is expected to perform initial design and development in Contractor-provided or commercially available environments prior to ATD issuance and CDC environment provisioning, or whether all development must occur within the CDC environment. If Contractor environments are used, please confirm the applicable security configuration and scanning requirements under Standard-3.

Yes, provided appropriate security configuration and scanning consistent with Standard-3 requirements are applied, and no Government/PHI/PII data is used prior to ATD/ATO issuance.

84. Please clarify whether the Contractor is expected to author the full ATD, SA&A, and ATO documentation packages, or to provide supporting input to CDC-authored packages. This materially affects the level of effort proposed.

Correct. No, the contractor will not need to obtain a separate ATO. However, the Contractor is responsible for updating the security and privacy documentation package.

85. Section G requires Key Personnel resumes to demonstrate hands-on experience in healthcare informatics, public health surveillance, or healthcare data management. Will comparable experience delivering secure, regulated data systems for other federal agencies be considered qualifying?

Yes, comparable experience delivering secure, regulated data systems for other federal agencies will be considered.

86. Which positions are considered Key Personnel, and is there a limit on the number of resumes? The CDCH.04 table is blank.

See answer to Question 33. There is no stated limit on the number of Key Personnel resumes.

87. Task 1.4 requires intelligent completion assistance. If an offeror proposes to meet this with rules-based logic and conditional guidance rather than artificial intelligence, is an AI Compliance and Risk Management Plan (Attachment 1) still required?

See answer to Question 31.

88. Are the Data Management Plan, AI Compliance and Risk Management Plan, Incident Response Plan, ACR/VPAT, draft subcontracting agreement, and three-year employee turnover table required with the proposal, and if so, are they excluded from the 15-page limit?

See answer to Question 3.

89. Please confirm CLIN 0001 is the only CLIN offerors price, and whether it is invoiced in nine equal monthly increments or upon deliverable acceptance.

Confirmed, CLIN 0001 is the only CLIN offerors are to price. Offerors are to include \$31,500 as the unit and NTE extended price for CLINs 0002 and 0003. The proposed payment schedule can be by deliverable acceptance or monthly increments.

90. Section 8 states final release is due at DOA + 150 days, but the stated period of performance is nine months. Please confirm the correct final release date.

See answer to Question 13.

91. Standard-11 requires open-source release under FAR 52.227-14, while FAR 52.227-17 restricts release without Contracting Officer permission. Which governs custom code developed under this order?

The custom code is to be governed by FAR 52.227-17.

92. Could CDC provide an overview of the current VAERS reporting application's architecture, including the existing application components, frameworks, databases, APIs, integrations, and infrastructure that the Contractor will need to interface with or replace?

See PWS Subsection D for the current architecture summary (Azure SQL Managed Instance, VMs, ColdFusion, Tomcat, Node.js, .NET, Akamai, Blob Storage, Key Vault, Logic Apps). Additional documentation will be furnished at kickoff.

93. Could CDC provide documentation of the current VAERS data model, data elements, business rules, validation rules, and integration specifications that will be available to the Contractor at project kickoff?

See answer to Question 32.

94. Could you please confirm will the Contractor be expected to modify or replace any existing VAERS backend components, or is the scope limited to developing the new reporting application and integrating with existing VAERS services?

Scope is limited to developing the new reporting application and integrating with existing VAERS services. Wholesale replacement of backend components is not required.

95. Could CDC clarify which Azure services, environments, subscriptions, networking components, CI/CD tools, and security services will be Government-furnished and which are expected to be provided/configured by the Contractor?

CDC furnishes the Azure environment, accounts, and standard platform services as described in PWS Subsection B. Contractor-specific development tooling and licensing not already available in the CDC environment are the Contractor's responsibility.

96. Could CDC identify the APIs/interfaces currently available for integration with VAERS and clarify whether new APIs or integration services are expected to be developed as part of this effort?

Interface specifications will be provided at kickoff per PWS Section 9. New integration services/structured data mappings may be required consistent with Task 1.9.

97. Could CDC provide the complete current VAERS form and identify the specific differences in fields, validation rules, and branching logic required between public and healthcare-provider reporting?

The current publicly available VAERS form may be used as a proxy for prototype development. Any additional data elements, validation rules, and branching specifications will be furnished at kickoff if not available publicly online.

98. Could CDC clarify whether the intelligent form assistance and supplemental-document recommendation capabilities are expected to use AI/ML/Generative AI, rules-based logic, or another approach?

See answer to Question 30.

99. Could CDC provide the expected reporting, usage metrics, and analytics requirements, including the specific metrics that must be captured for public and healthcare-provider users?

Specific metric definitions will be finalized with CDC after award; see PWS Task 1.10 and PRS#4 for baseline performance targets.

100. What is the approved budget or range allocated for this project?

[See answer to question 57.](#)

101. Is CDC open to an offshore-blended delivery model, and are there any data residency or US-only staffing restrictions?

[CDC is generally open to an offshore-blended delivery model, provided that the model meets CDC's security, compliance, data protection, and service-level requirements. There are no general staffing restrictions; however, any sensitive or regulated CDC data must remain within approved environments and jurisdictions.](#)

102. Please confirm whether the required Data Management Plan and AI Compliance and Risk Management Plan, if applicable, are included within the 15-page Technical Proposal limitation or may be submitted as separate attachments.

[See answer to question 3.](#)

103. Please clarify whether the Accessibility Conformance Report (ACR/VPAT) is required at proposal submission or is a post-award deliverable.

[See answer to Question 39.](#)

104. Does CDC expect the "intelligent form assistance" and supplemental document suggestion capabilities to use AI/GenAI, or will a rules-based approach be considered acceptable?

[See answer to Question 30.](#)

105. If AI functionality is expected or permitted, which CDC-approved AI services, models, or technologies may be used within the CDC-managed environment?

[Government approved FedRAMP-authorized AI services, with CDC approval, are permitted. These may include, but not be limited to OpenAI, Claude, Gemini and MS Copilot.](#)

- 106. Will CDC provide the required Azure infrastructure and cloud services, or should offerors include Azure infrastructure, storage, monitoring, and other cloud consumption costs in the firm-fixed-price proposal?

[CDC provides the underlying Azure environment; however, any Contractor-specific consumption, storage, or licensing beyond the standard CDC-furnished environment should be identified and priced as a separate IT supply line item.](#)

107. Can CDC confirm whether the production-ready Final Release at DOA +150 days represents the production deployment/go-live date, and clarify the scope of work expected during the remaining period of performance?

[See answer to Question 13.](#)

108. Is CDC prescribing a technology stack for the new application, or may the contractor propose an appropriate .NET, Node.js, Java, Power Platform, or other Azure-compatible architecture?

[No specific stack is prescribed; offerors may propose an appropriate Azure-compatible architecture.](#)

109. For the low-code/configurable interface, is CDC expecting a specific platform, or would a custom administrative interface that allows program personnel to manage content and configuration satisfy the requirement?

[No specific platform is mandated. A custom administrative interface satisfying Task 1.8's intent is acceptable.](#)

110. Does the supplemental-document suggestion tool require artificial intelligence or machine learning, or may it use deterministic rules and configurable business logic?

[See answer to Question 30.](#)

111. If the offeror proposes no AI use, is an affirmative no-AI disclosure sufficient, or must the AI Compliance and Risk Management Plan attachment still be completed and submitted?

[See answer to Question 31.](#)

112. What level of functionality is expected in the proposal-stage prototype? Would an interactive prototype using synthetic data be acceptable, or must it contain working application code?

[See answer to Question 28.](#)

113. Is there a required minimum staffing level or number of key personnel? May one qualified individual perform multiple key roles?

[There is no minimum staffing level. Yes, one individual can perform multiple roles.](#)

114. Must an ACR/VPAT or Section 508 conformance checklist be submitted with the proposal, or is it only a post-award deliverable?

[See answer to Question 23.](#)

115. Will CDC provide security, ATD, privacy and integration-documentation templates, along with access to Government vulnerability-scanning tools?

[Available templates will be furnished at kickoff. Government scanning tools/access will be addressed as part of Azure environment provisioning; Contractor tooling not otherwise furnished should be priced by the Contractor.](#)

116. What are the anticipated transaction volume, concurrent-user volume and existing performance baselines?

Anticipated transaction volume (annual): 2,256,501 site visits and 22,016 report submissions (2025 observed).

Anticipated transaction volume (daily average): 6,183 visits/day and 60 reports/day.

Concurrent-user volume: With an assumed 15-minute average session and uniform traffic, average concurrent users ≈ 64 ; real peak concurrent users may be several times higher and require hourly/minute logs to determine.

Existing performance baselines: not available.

117. For Task 2.3, please clarify whether the supplemental-document suggestion capability is expected to analyze information contained within the VAERS submission workflow only, or whether the Government expects the solution to analyze and/or abstract the contents of uploaded medical records in order to generate document suggestions.

The tool is expected to analyze information within the submission workflow to suggest supplemental documents; deep content abstraction of uploaded medical records is not required.

118. Please confirm that the proposal prototype is not expected to replicate all production VAERS business rules, field-level validation rules, and integration mappings that will be provided by the Government at kickoff, and that those Government-furnished requirements will be incorporated and finalized following award.

Confirmed. See answer to question 28 also.

119. For the Working Prototype (Alpha) Demonstration Build due within 45 calendar days after award, please confirm whether CDC expects the Alpha to be deployed within the CDC-managed Azure environment, or whether the Alpha may initially be demonstrated in a Contractor-controlled development environment while ATD, onboarding, and CDC environment provisioning activities are completed.

See answers to Questions 55 and 83.

120. Please clarify the Government's expected scope of Contractor effort between acceptance of the Production-Ready Final Release at DOA + 150 days and the end of the stated nine-month period of performance. In particular, should offerors assume this period is intended for stabilization, production support, defect remediation, authorization-related activities, transition, or additional development?

See answers to Questions 13.

121. PWS §4, Task 1.8 – Low-Code/Configurable Interface
For the “low-code/configurable interface,” does CDC have a preferred or mandated platform or technology, or may the Contractor propose the most appropriate configurable content/interface management approach within CDC Azure?

See answer to Questions 109

122. PWS §4, Task 1.9 / §9 – System Integration

Please identify the expected integration method(s) between the modernized application and existing VAERS systems. Should Offerors assume API, database, file-based, messaging, or other interfaces, and will interface specifications be provided at kick-off?

[See answers to Questions 96.](#)

123. PWS §4, Task 1.9 – Downstream VAERS Systems

Will the Contractor be responsible only for producing VAERS-compatible structured data, or also for modifying existing downstream VAERS applications, databases, interfaces, or workflows to receive the new submissions?

[The Contractor is responsible only for producing VAERS-compatible structured data; modification of downstream VAERS applications is out of scope.](#)

124. PWS §1.D – Existing Documentation

Will CDC provide existing architecture documentation, database schemas, interface specifications, VAERS data dictionaries, validation rules, source-code documentation, and current business rules at kick-off?

[See answer to Questions 32.](#)

125. PWS §8 – Government-Furnished Environment and Access

The solicitation requires a Working Prototype within 45 days and a Beta Release within 90 days. Please confirm whether CDC anticipates that the required Azure environments, accounts, networking, repositories, credentials, and development access will be available immediately upon award.

[See answer to Questions 22.](#)

126. PWS §3 / §8 – Government Dependencies and Schedule

If CDC-provided cloud access, integration information, security approvals, Government reviews, or other Government dependencies delay performance, will corresponding deliverable dates be adjusted?

[Yes, the CDC will work with the contractor to update the deliverable due dates as needed for any Government related delays.](#)

127. PWS §3.4 / Security Requirements – Security Tools

Will CDC provide approved vulnerability-scanning, SAST/DAST, code-scanning, and security-testing tools within the development environment, or should the Contractor include its own tool licensing in the Firm-Fixed-Price proposal?

[See answer to Questions 115.](#)

128. CDCH.10 – AI Compliance and Risk Management Plan

If an Offeror proposes AI or generative AI only as an optional enhancement, must an AI Compliance and Risk Management Plan be submitted with the initial quotation, or only if AI is included in the proposed base technical solution?

[See answer to Questions 31.](#)

129. Task 1.10 – Median Submission Time

For the median end-to-end submission time target of ≤ 10 minutes, will CDC define a representative “typical report” and Government test scenario for measurement purposes?

[A representative test scenario will be defined in coordination with the Contractor after award.](#)

130. Section 8 – Final Release Schedule

The deliverables table states that Final Release is due “by end of PoP (DOA + 150 days),” while the stated period of performance is approximately nine months. Please clarify whether production-ready Final Release is due 150 days after award or at the end of the nine-month period of performance.

[See answer to Questions 13.](#)

131. Existing Support Contracts

Please identify the roles and scope boundaries of existing VAERS contractors with whom the awardee will be required to coordinate, particularly responsibility for current VAERS databases, legacy applications, integration interfaces, infrastructure, and production operations.

[See answer to Questions 2.](#)

132. The PWS primarily identifies Contractor support for issuance of an Authority to Develop (ATD), while Section 11 and the applicable security requirements also reference obtaining an Authority to Operate (ATO). Please clarify whether the Contractor is responsible for supporting both ATD and ATO issuance within the nine-month period of performance and identify the authorization milestones required before Beta and Final/Production deployment.

[See answer to Questions 9.](#)

133. The existing VAERS environment operates within CDC-managed Azure. Please clarify whether the existing VAERS Azure environment and supporting infrastructure are currently covered by an active CDC ATO and whether the new application is expected to inherit or leverage existing security controls and authorization artifacts, or require a separate/new authorization package.

[The existing VAERS Azure environment operates under CDC's existing authorization; the new application will be evaluated for inheritance of applicable controls where appropriate, subject to CDC/OCIO's determination during the ATD/ATO processes. The Contractor is responsible for updating the security and privacy documentation package.](#)

[See answer to Question 9.](#)

134. The Technical Proposal is limited to 15 pages, while the Prototype is submitted through an external accessible link. Please confirm that the Prototype itself and content presented through the external Prototype link are excluded from the 15-page Technical Proposal limitation. Please also clarify whether supporting Prototype documentation submitted within the Technical Volume counts toward the page limit.

[See answer to Question 3.](#)

135. The PWS requires coordination with other existing contracts supporting VAERS and with CDC/OCIO hosting teams. Please identify the anticipated roles and responsibilities of the existing VAERS support contractors and CDC/OCIO teams, particularly regarding legacy-system maintenance, data interfaces, infrastructure, security authorization, deployment, and production support.

[See answer to Questions 2.](#)

136. The overall period of performance is September 28, 2026 through June 27, 2027, while the Final Production-Ready Release is identified as due "By end of PoP (DOA + 150 days)." Please clarify whether the Final Release is required at DOA + 150 calendar days or at the end of the nine-month period of performance. If Final Release is required at DOA + 150 days, please identify the principal activities expected during the remaining period of performance.

[See answer to Question 13. The remaining period is intended for stabilization, production support, defect remediation, and transition-related activities.](#)

137. The RFQ states that a Data Management Plan (DMP) shall be submitted with the proposal for applicable public-health data collection/generation activities and that omission of a required DMP will render a proposal unacceptable. However, the proposal-volume instructions do not identify where the DMP is to be submitted. Please confirm whether a DMP is required with this quotation and, if so, identify the applicable volume/tab and whether the DMP is excluded from the 15-page Technical Proposal limitation.

[See answer to Question 3.](#)

138. The solicitation provides a template for the AI Compliance and Risk Management Plan when planned or potential AI use is identified; however, Section F does not appear to specify where the completed Plan should be submitted. Please confirm whether the completed AI Compliance and Risk Management Plan should be submitted as a separate document/attachment and identify the applicable Volume/Tab for submission. Please also confirm that the Plan is excluded from the 15-page Technical Proposal page limitation.

[See answer to Question 3.](#)

139. Are there any specific positions that must be included as Key Personnel, and if not, may Offerors determine their own Key Personnel positions?

[See answer to Questions 33.](#)

140. Can a signed teaming agreement that clearly defines the subcontractor's scope, duties, and workshare satisfy the requirement for a proposed draft Subcontracting Agreement?

Yes.

141. Do the required Data Management Plan (DMP) and, if applicable, the AI Compliance and Risk Management Plan count against the 15-page Volume I Technical Proposal limit, or are they submitted as separate, uncounted attachments? Neither is listed among the four Volume I tabs, but a proposal without a DMP is deemed "Unacceptable."

[See answer to Question 3.](#)

142. For the required prototype submitted "through an accessible electronic link," can it sit behind a standard reviewer login (e.g., a staging URL or design-tool link), or must it be fully public and unauthenticated? Are there size, duration, or browser/device constraints the evaluators will test against?

[See answer to Question 28.](#)

143. The position sensitivity designation table referenced in the security requirements is blank in the posted RFQ. Can CDC confirm the specific investigation tier(s) required for contractor staff, given the roster must be submitted 30 days before the contract effective date?

[Public Trust Tier 2.](#)

144. What is CDC's anticipated timeline for issuing PIV cards/laptops and granting Azure environment access / the Authority to Develop (ATD), relative to the Task 1 milestones (wireframes at DOA+21, technical design at DOA+30, Alpha prototype at DOA+45)?

[See answer to Questions 13.](#)

145. Can CDC identify the existing contract(s) supporting the current VAERS system and clarify the scope boundary between that support and this new development effort? Will the current VAERS codebase and technical documentation be made available to offerors before or only after award?

[See answer to Questions 2. Yes, current codebase and tech documents will be made available at kick-off.](#)

146. Should the optional Phase 2 medical-image upload capability be priced now as a separate option CLIN in this proposal, or is it intended to be priced later via a future modification, outside the scope of this Price Proposal?

[See answer to question 13.](#)

147. CLIN 0001's unit price/amount fields are blank and no total estimated value, budget range, or IGCE is stated in the solicitation. Can CDC share an estimated value range or historical VAERS support spend to help size the technical approach appropriately?

[See answer to question 57.](#)

148. For the Similar Experience factor, must qualifying past performance be with a federal public-health surveillance system specifically, or will comparable state-level, commercial, or academic healthcare-data-system experience be weighted equally?

[See answer to Questions 7.](#)

149. Cloud Environment - Is Azure Kubernetes Service (AKS) service available in the cloud environment?

[Yes, the full list of FedRAMP compliant products available as part of the Azure and AWS FedRAMP Audit Scopes may be considered for usage.](#)

150. CDCH.04 lists an unpopulated Key Personnel table with no titles specified, while Section F, Tab 3-2 requires resumes for "all proposed Key Personnel identified by the Offeror." Does CDC have specific required Key Personnel roles in mind for this task order (e.g., Program Manager, UX/Technical Lead, Security/ATD Lead), or is the Offeror free to designate its own Key Personnel roles for evaluation purposes?

[See answer to Question 33.](#)

151. Section 9 (Reference Materials) states that Government-provided VAERS data element definitions, business rules, and integration requirements will be provided "at kickoff" — after award. But Task 1.6 and the Tab 2-2 Prototype evaluation require the Offeror to design a branching-logic reporting form "using the data elements of the current VAERS form" before award. Will CDC release the current VAERS data element list and business rules prior to the September 4 due date, or should Offerors prototype against the current public-facing VAERS form as a proxy?

[See answer to Questions 79 and 97.](#)

152. Question and proposal submission email address. Section F's General Instructions direct the complete proposal to uw08@cdc.gov (letter "o"), then in the very next sentence direct "all questions, requests for clarification, and other inquiries" to uw08@cdc.gov (digit "0") — a single character apart. Page 1 of the SF-18 and the SAM.gov listing both use the letter-"o" form. Please confirm, in writing, the single correct address for proposal submission.

[See answer to Question 16.](#)

153. Placement of the Data Management Plan, AI Compliance and Risk Management Plan, and Incident Response Plan. CDCL.10 makes a Data Management Plan mandatory ("a proposal received without a DMP will be deemed 'Unacceptable'"); CDCL.03/CDCH.10 requires an AI Compliance and Risk Management Plan "using the template provided" whenever AI use is planned, expected, or possible; and a Standard-4 deliverables table lists an Incident Response Plan as due "as part of Proposal." None of the three appears in Section F's Volume I tab structure (Tabs 1–4) or Volume II price tab, and none is named among the stated exclusions from the 15-page Technical Proposal limit (only cover pages, tables of contents, and resumes are excluded). Please confirm: (a) which of these documents must be submitted with the proposal and where it/they should be placed relative to the required tab structure and (b) confirm that each is excluded from the 15-page limit OR consider increasing the page limit substantially to accommodate any additional information compiled in the required document(s).

See answer to Question 3.

154. Prototype (Tab 2-2) documentation and the page limit. Tab 2-2 must be “submitted through an accessible electronic link,” a format Section F’s paged format rules (page count, font, margins) do not account for. Section F never states whether written material accompanying the prototype link (e.g., an orientation guide or requirements cross-reference) counts toward the 15-page Technical Proposal limit. Please confirm whether any prototype-related written material is included in or excluded from the 15-page limit.

See answer to Question 3.

155. Tabbing. Please confirm whether, for the purposes of Volume 1 – Technical Proposal compilation, tabs and section headings can be used interchangeably. Section F, Subsection II specifies that the electronic Technical Proposal be organized across multiple tabs (e.g., Tab 1, Tab 2-1, Tab 2-2, Tab 3-1, Tab 4) within a strict 15-page limit. In an electronic MS Office Word or PDF submission, treating each tab as a separate document section requiring a hard page break or dedicated digital divider sheet significantly consumes the page count. Please confirm that offerors may flow content continuously using clear inline section/tab headings and PDF bookmarks, or clarify whether electronic divider/tab pages are exempt from the 15-page limitation.

Tabs and section headings can be used interchangeably. Offerors may flow content continuously using clear inline section/tab headings and PDF bookmarks.

156. Personnel list submission timing — direct conflict. The Standard-1 deliverables table requires the list of personnel with defined roles “within 7 days” (before an employee begins working), while the Standard-4 deliverables table states “within 10 days” for what appears to be the same deliverable. Please reconcile these two timeframes.

Within 7 days.

157. Travel CLIN inconsistent with the PWS’s no-travel statement. PWS Section 7 states that “no travel, other than that necessary for employee onboarding, is anticipated,” yet CLIN 0003 carries a \$31,500 NTE estimate for non-conference meeting travel. Please confirm (a) that CLIN 0003 is a Government-directed contingency that offerors should nonetheless include in their price proposal, and (b) that both travel CLINs 0002 and 0003 are excluded from the price evaluation.

Per Section 7: No travel, other than that necessary for employee onboarding, is anticipated for this contract. The Contractor shall perform work remotely and coordinate with CDC via virtual meetings. If the Government requests travel (e.g., for in-person meetings at CDC Atlanta), travel must be approved in advance by the COR and shall be reimbursed in accordance with applicable Federal Travel Regulation (FTR) requirements and the travel terms of the contract.

158. Pass/fail evaluation gates with no corresponding submission instruction. CDCM.03 and CDCM.04 make the Data Management Plan and the AI Compliance and Risk Management Plan pass/fail gates — failing either eliminates a proposal outright, with no opportunity to cure since award is intended without discussions — yet Section F’s stated proposal-contents instructions never direct offerors to prepare or submit either document. An offeror following Section F’s

content instructions alone would have no notice that these outcome-determinative gates exist. Please confirm Section F or Section G will be amended to obviate this deficiency.

The Data Management Plan and the AI Compliance and Risk Management Plan are to be submitted as part of the Technical Proposal as attachments. They are not included within the established page limit. The solicitation will be amended.

159. Three-year employee turnover table required only in the evaluation criteria. Section G's Tab 3-1 evaluation criteria require "a table showing employee turnover rates for each of the past three years," but Section F's Tab 3-1 instructions ask only for the offeror's approach to "rapidly obtaining and replacing qualified staff" and "availability of qualified personnel," with no mention of a turnover table. An offeror preparing Tab 3-1 solely from Section F would omit a document Section G scores. Please confirm the turnover table is not required and that Section F will be amended to state this explicitly.

Section G language to include "a table showing employee turnover rates for each of the past three years" is applicable. Solicitation Section F will be amended.

160. Incident Response Plan is a required deliverable that the evaluation criteria never mention. A Standard-4 deliverables table lists an Incident Response Plan as due "as part of Proposal," but Section G's Technical Proposal evaluation criteria (Tabs 1–4) do not reference it, and it is not one of the two named pass/fail gates (DMP, AI Plan). If the Incident Response Plan is indeed required with the proposal (see A.2 above), please clarify whether the Incident Response Plan is evaluated at all — and if so, under which factor or sub-factor and against what criteria — or confirm it is a pass/fail administrative submission outside the tradeoff, consistent with the treatment of the DMP and AI Plan.

The Incident Response Plan is within the Information Security and Privacy requirements tied to any product delivered, specifically it is part of Standard 4 when Implementing Cloud Services. Security and Privacy requirements are not evaluated as part of the proposal but must be taken account in the design of delivery and have specific deliverables to be able to deploy.

161. No disclosed rating methodology for the Technical, Management, and Similar Experience sub-factors. Section G states the DMP and the AI Compliance and Risk Management Plan will each be rated "Pass/Fail," but does not state whether the three Factor 1 sub-factors (Technical Approach, Management Approach, Similar Experience) will be rated adjectivally (e.g., Outstanding/Good/Acceptable/Marginal/Unacceptable), numerically, or narratively only, for purposes of the best-value tradeoff. Please confirm the rating methodology that will be used to score these sub-factors.

The Government is not required to disclose the rating methodology.

162. Internally inconsistent statement of Price's relative importance. FAR 52.212-2(a), incorporated in Section E, lists the evaluation factors "in descending order of importance" as (1) Technical, (2) Price, (3) Past Performance — literally ranking Price above Past Performance. CDCM.01 separately states that all non-price factors combined are significantly more important than price, and that price becomes determinative only if technical proposals are judged essentially equal. These describe different relationships between Price and Past Performance. Please confirm how Price and Past Performance rank relative to each other outside of an essentially-equal Technical result.

FAR 52.212-2 properly lists the evaluation factors in descending order of importance:

Factor 1: Technical

Sub-Factor 1: Technical Approach

Sub-Factor 2: Management Approach

Sub-Factor 3: Similar Experience

Factor 2: Price

Factor 3: Past Performance

Evaluation factors other than price when combined are significantly more important than price.

Individually, Technical is more important than Price and Price is more important than Past Performance. However, combined Technical and Past Performance are more important than Price.

CDCM.01 is removed from the solicitation.

163. Option pricing is contemplated for evaluation but absent from the pricing preparation instructions. FAR 52.212-2(b) states the Government will evaluate offers “by adding the total price for all options to the total price for the basic requirement,” and FAR 52.217-8 gives the Government a six-month option to extend services. The Section B Schedule contains no option CLIN, and Section F's price-volume instructions never direct offerors to submit option pricing. Please confirm whether offerors must price the six-month extension option for evaluation purposes, and if so, in what CLIN structure and format.

[See answer to question 13](#)

164. Can the government please provide a higher resolution version of the architectural diagram for the current architecture that is provided in Subsection D?

[VAERS Architecture 01_07_2025.pdf](#)

165. Does the low-code/configurable interface envisioned apply to static content only (landing page copy, FAQs, tooltips) or does the government also expect program personnel to make direct edits to the submission form itself, including fields or branching logic?

[Low-code solution should apply to both static and submission form, including fields and branching logic.](#)

166. Does the low-code/configurable interface envisioned apply to static content only (landing page copy, FAQs, tooltips) or does the government also expect program personnel to make direct edits to the submission form itself, including fields or branching logic?

[See answer to question165.](#)

167. Will the government allow research with potential users of the VAERS system to allow for user feedback on user flows, wireframes, and clickable prototypes?

[Yes.](#)

168. Can the government share the data model for VAERS?
[No, however, a data dictionary will be provided.](#)
169. Does CDC have access to a customer satisfaction measuring tool for VAERS through an existing enterprise license?
[No.](#)
170. Should CLIN 0001 pricing in contractor's Volume II response include Phase 2 estimated pricing or will this pricing be handled after Phase 1 is completed?
[No. Any additional work will be addressed in a future solicitation.](#)
171. Does the government intend to specify the key personnel in the RFQ? No Key Personnel are identified in CDCH.04 but resumes for Key Personnel are required as part of the response.
[See answer to Question 33.](#)
172. The PoP is 9/28/2026 - 6/27/2027 which is approximately 9 months. The deliverables table in Section 8 lists the Testing and Final release as DOA + 150 days. 150 days is approximately 5 months. Can the government clarify what the remaining 4 months might include?
[See answer to Question 13.](#)
173. The pricing schedule lists \$31,500 for "meeting travel" (CLIN 0003). Section 7 of the PWS says the only travel expected is for onboarding. Can the government clarify whether non-onboarding-related travel is expected?
[Per Section 7: No travel, other than that necessary for employee onboarding, is anticipated for this contract. The Contractor shall perform work remotely and coordinate with CDC via virtual meetings. *If the Government requests travel \(e.g., for in-person meetings at CDC Atlanta\), travel must be approved in advance by the COR and shall be reimbursed in accordance with applicable Federal Travel Regulation \(FTR\) requirements and the travel terms of the contract.*](#)
174. The government indicates that a template should be used to document an AI Compliance and Risk Management Plan. The reference indicates in accordance with CDCL.10, but that section refers to a data management plan. Please confirm the correct template that should be used for the AI Compliance and Risk Management Plan.
[The DMP is an example template. Contractor is encouraged to submit a similar plan for AI compliance and risk management.](#)
175. PWS Task 1.10 requires a reduction in submission abandonment rate of at least 30% "compared to the baseline established at project kickoff." Will the Government provide current abandonment rate, median submission time, and page load time data at kickoff, or is the Contractor expected to instrument the existing application and derive the baseline? If Contractor-derived, what measurement window will the Government consider acceptable for establishing that baseline?

[See answer to Question 44.](#)

176. Will the Government confirm the method of verification for the Task 1.10 performance targets (page load ≤ 3 seconds, median end-to-end submission ≤ 10 minutes, abandonment reduction $\geq 30\%$), and at what point in the period of performance those targets are assessed for acceptance?

[See answer to Question 13.](#)

177. PWS Task 4 is identified as Transition Out. However, the Section 8 Deliverables/Reporting Schedule assigns to "Task 4" the Security and Privacy Documentation Package Supporting ATD Issuance, the Compliance Documentation Package (Security/Privacy/Section 508), and the Accessibility Conformance Report (ACR/VPAT) and Remediation Plan. These deliverables appear to derive from PWS Tasks 3.3, 3.5, and 3.6 rather than from Transition Out. Will the Government confirm the correct task mapping for these three deliverables, and clarify whether the "PM / Task 4" designation for the Project Plan indicates a distinct project management workstream?

[Thank you for identifying this incongruence around the security package. You are correct these Deliverables align with Task 3 and will also be required for any Transition Out to ensure all documents are with the government.](#)

178. PWS Task 4.4 requires submission of a detailed Transition-out Plan for COR approval at least two months prior to contract end. This deliverable does not appear in the Section 8 Deliverables/Reporting Schedule. Will the Government confirm that the Transition-out Plan is a required deliverable and identify its due date, and confirm whether the monthly transition-readiness assessments required by Task 4.1 are separately deliverable to the COR?

[Yes, a transition out plan is a deliverable and should be provided as outlined in the PWS Task 4.4 two months prior to contract end. Yes, the monthly transition-readiness assessments required by Task 4.1 should be delivered to the COR.](#)

179. The Section 8 Deliverables/Reporting Schedule lists Testing and Final Release (Production-Ready) as due "By end of PoP (DOA + 150 days)." For an award dated 28 September 2026, DOA + 150 days falls in late February 2027, while the period of performance extends to 27 June 2027. Will the Government confirm which date governs final release, and clarify the scope of contractor effort expected between final release and the end of the period of performance — specifically whether that interval covers sustainment, warranty, defect remediation, transition-out activities, or a combination?

[See answer to Question 13.](#)

180. Regarding PWS Task 2.4.2, should offerors exclude Phase 2 (upload of pictures and medical images) entirely from the quoted price, or does the Government wish to see a separate, non-binding level-of-effort estimate for Phase 2 for planning purposes?

Offerors should exclude Phase 2 from the quoted price. A non-binding, separately identified LOE estimate for planning purposes may be included but is not required.

181. PWS Task 2.3 requires a tool that "scans healthcare-provider submissions and suggests supplemental documents to upload." Does the Government consider this an artificial intelligence or machine learning capability, or would a deterministic, rules-based implementation satisfy the requirement?

A deterministic, rules-based implementation is acceptable; AI/ML is not mandated.

182. Will the Government provide interface documentation for the existing VAERS system — including current data formats, transmission mechanisms, and integration endpoints — and will that documentation be made available during the question period or at kickoff?

This documentation will be furnished at kickoff.

183. Does CDC have a preferred or previously authorized low-code platform within the CDC-managed Azure environment for the configurable interface required by PWS Task 1.8, or is platform selection at the Contractor's discretion subject to ATD approval?

No specific platform is mandated; selection is at the Contractor's discretion, subject to ATD/security approval.

184. 10. Given that the Beta Release is due within 90 calendar days after date of award, what is CDC's typical elapsed time from Contractor submission of the security and privacy documentation package to issuance of the Authority to Develop? Will the Government consider ATD processing time as excusable delay if it exceeds the Contractor's control?

Historical median processing time is not available for release. ATD processing delays attributable to the Government would be treated consistent with standard FAR excusable-delay provisions.

185. What is the anticipated elapsed time from award to issuance of PIV cards and delivery of Government-furnished laptops?

This timeline is not specified in the solicitation; offerors should build reasonable ramp-up time into their schedules and expect adjustment for Government-caused delays.

186. Does the Government impose page limits, a required volume structure, or a required file format for the technical quotation?

See solicitation Section F.

187. Is a completed Accessibility Conformance Report (ACR/VPAT) required with the quotation, or only as a deliverable during performance as indicated in the Section 8 schedule?

An HHS Section 508 Accessibility Conformance Checklist is required at proposal stage; the full ACR/VPAT is a post-award deliverable due with Beta Release, updated at Final.

188. Attachment 1, Artificial Intelligence (AI) Use Compliance and Risk Management Plan, states it applies where "the primary deliverable is not the development or delivery of an AI system." If the Contractor uses AI-assisted software development tooling in internal operations only, is Attachment 1 required to be submitted with the quotation, or completed upon award?

Yes, the AI Compliance and Risk Management Plan is required with every proposal regardless of the nature or scope of proposed AI use, including internal-only tooling.

189. The PWS primarily identifies Contractor support for issuance of an Authority to Develop (ATD), while Section 11 and the applicable security requirements also reference obtaining an Authority to Operate (ATO). Please clarify whether the Contractor is responsible for supporting both ATD and ATO issuance within the nine-month period of performance and identify the authorization milestones required before Beta and Final/Production deployment.

See answer to Question 9.

190. The existing VAERS environment operates within CDC-managed Azure. Please clarify whether the existing VAERS Azure environment and supporting infrastructure are currently covered by an active CDC ATO and whether the new application is expected to inherit or leverage existing security controls and authorization artifacts, or require a separate/new authorization package.

The existing VAERS Azure environment operates under CDC's existing authorization. The new application will be evaluated for inheritance of applicable controls where appropriate, subject to CDC/OCIO's determination during the ATD/ATO process.

191. The Technical Proposal is limited to 15 pages, while the Prototype is submitted through an external accessible link. Please confirm that the Prototype itself and content presented through the external Prototype link are excluded from the 15-page Technical Proposal limitation. Please also clarify whether supporting Prototype documentation submitted within the Technical Volume counts toward the page limit.

See answer to Question 3.

192. The PWS requires coordination with other existing contracts supporting VAERS and with CDC/OCIO hosting teams. Please identify the anticipated roles and responsibilities of the existing VAERS support contractors and CDC/OCIO teams, particularly regarding legacy-system maintenance, data interfaces, infrastructure, security authorization, deployment, and production support.

[Coordination points and role divisions with existing VAERS support contractors and CDC/OCIO hosting teams will be identified after award.](#)

193. The overall period of performance is September 28, 2026 through June 27, 2027, while the Final Production-Ready Release is identified as due "By end of PoP (DOA + 150 days)." Please clarify whether the Final Release is required at DOA + 150 calendar days or at the end of the nine-month period of performance. If Final Release is required at DOA + 150 days, please identify the principal activities expected during the remaining period of performance.

[The remaining period is intended for stabilization, production support, defect remediation, and transition-related activities.](#)

194. The RFQ states that a Data Management Plan (DMP) shall be submitted with the proposal for applicable public-health data collection/generation activities and that omission of a required DMP will render a proposal unacceptable. However, the proposal-volume instructions do not identify where the DMP is to be submitted. Please confirm whether a DMP is required with this quotation and, if so, identify the applicable volume/tab and whether the DMP is excluded from the 15-page Technical Proposal limitation.

[See answer to Question 3.](#)

195. The solicitation provides a template for the AI Compliance and Risk Management Plan when planned or potential AI use is identified; however, Section F does not appear to specify where the completed Plan should be submitted. Please confirm whether the completed AI Compliance and Risk Management Plan should be submitted as a separate document/attachment and identify the applicable Volume/Tab for submission. Please also confirm that the Plan is excluded from the 15-page Technical Proposal page limitation.

[Submit as a standalone attachment \(Word or PDF\), outside the four Volume I tabs. It is excluded from the 15-page Technical Proposal limit.](#)

196. If subcontractors are proposed, the RFQ requires a written copy of the proposed draft Subcontracting Agreement. Please confirm whether the draft Subcontracting Agreement is excluded from the 15-page Technical Proposal limitation and may be submitted as an attachment.

Yes, it may be submitted as a separate attachment and is excluded from the 15-page limit.

197. Are there any specific positions that must be included as Key Personnel, and if not, may Offerors determine their own Key Personnel positions?

See answer to question 33. Offerors may propose their own Key Personnel positions consistent with their technical and management approach.

198. Can a signed teaming agreement that clearly defines the subcontractor's scope, duties, and workshare satisfy the requirement for a proposed draft Subcontracting Agreement?

Yes.

199. The RFQ states that cover pages, tables of contents, and resumes are excluded from the 15-page Technical Proposal limitation. Please confirm whether Tab 4 (Similar Experience/Past Performance) content, including project summary sheets or past performance reference forms, is also excluded from the page count.

See answer to Question 3.

200. The RFQ states that cover pages, tables of contents, and resumes are excluded from the 15-page Technical Proposal limitation. Please confirm whether Tab 4 (Similar Experience/Past Performance) content, including project summary sheets or past performance reference forms, is also excluded from the page count.

See answer to Question 3.

201. The RFQ imposes a 12 MB attachment limit and directs offerors to divide submissions across multiple emails, while stating that transmission of an email does not by itself establish Government receipt. Will the Government acknowledge receipt of each proposal email, or is there another mechanism by which offerors can confirm complete and timely receipt?

The Government will acknowledge receipt of each proposal by email.

202. Volume II, Tab 1 is titled "Price Breakdown Worksheet," but no worksheet or pricing template is included in the RFQ package. Will the Government provide a required pricing format, or may offerors submit their own format addressing the cost elements?

Offerors may submit their own format addressing the cost elements.

203. Section E incorporates several provisions requiring representations and certifications, but Section F does not address them. Are completed representations and certifications required to be submitted with the proposal, and if so, in which volume should they be included? Are they excluded from the 15-page Technical Proposal limitation?

Any representations and certifications required by Section E can be included with the Price Proposal. The Price Proposal does not have a page limitation.

204. PWS Section 8 requires Testing and Final Release "by end of PoP (DOA + 150 days)," while CLIN 0001 establishes a period of performance of 09/28/2026 through 06/27/2027, a span of approximately 273 days. Which governs for purposes of the Final Release delivery date?

[See answer to Question 13.](#)

205. FAR 52.217-8 is incorporated in full text and permits up to 6 additional months of performance, and Section E states that option prices will be added to the basic requirement for evaluation purposes. However, no option CLIN appears in Section B. Will the government calculate that themselves or should offerors price the 52.217-8 option period, and if so, in what format?

[See answer to question 13](#)

206. Page 61 excludes only cover pages, tables of contents, and resumes from the 15-page Technical Proposal limitation. The Data Management Plan and the AI Compliance and Risk Management Plan are both separately required and separately evaluated on a pass/fail basis. Can you confirm these two plans are excluded from the 15-page limitation?

[See answer to Question 3.](#)

207. Page 61 requires Technical Proposal text in Times New Roman 12-point, and separately establishes minimum font sizes of 8-point for graphs and illustrations and 10-point for "tables and text." Please clarify whether 10-point font is acceptable for narrative text presented within tables and graphics, or whether the 12-point requirement governs all narrative content.

[10-point font for narrative text is the acceptable minimum.](#)

208. Does the Government require a specific file naming convention for proposal files and emails beyond the sequencing identification described on page 61?

[No.](#)

209. Does supporting documentation accompanying the prototype count against the 15-page Technical Proposal limitation, or is the prototype and its accompanying documentation evaluated entirely outside the page limit?

[No, provide it as an attachment.](#)

210. Tab 2-2 requires a prototype demonstrating the proposed solution "to the maximum extent practicable." Is the Government expecting a functional, deployed web application, or is a representative demonstration of the proposed approach acceptable? What does the Government consider the minimum acceptable level of fidelity?

[Representative application is fine. However, see answer to question 28.](#)

211. The PWS requires branching logic using the data elements of the current VAERS form. Must the prototype use actual VAERS data elements and the current form structure, or is a representative reporting workflow sufficient for evaluation purposes?

[Representative application is fine. However, see answer to question 28.](#)

212. PWS Section 9 states that VAERS data element definitions, business rules, and integration requirements will be provided at kickoff. Can any of this information be released

during the solicitation period to inform prototype development, or should offerors develop prototypes based solely on the publicly available VAERS form?

[See answer to question 264.](#)

213. Will Government evaluators access the prototype link from CDC networks and CDC-managed devices? Are there any constraints on hosting platform, authentication method, or supported browsers that offerors should accommodate to ensure the prototype is accessible to evaluators?

[See answer to question 28.](#)

214. For how long after the proposal due date must the prototype link remain live and accessible to the Government?

[60 days. See answer to question 28.](#)

215. CDCH.10 requires the offeror to identify whether planned or potential AI use may involve a high-impact AI use case as defined in Appendix A of OMB M-25-21. Does CDC have a preliminary determination as to whether AI use in this application would constitute a high-impact use case?

[No, it would depend on how it would be used and employed within the system.](#)

216. CDCH.10 states that the Government will issue approval or disapproval before AI can be incorporated. What is the expected Government turnaround for that approval, and will an AI Compliance and Risk Management Plan found acceptable during proposal evaluation carry forward at award without requiring a second approval cycle?

[The AI Compliance and Risk Management Plan will be evaluated during the proposal evaluation phase. If approved, a second approval cycle is not required.](#)

217. The PWS requires the Contractor to coordinate with "the other existing contracts supporting the VAERS system." Which existing contractors will the awardee be required to coordinate with, and what mechanism will CDC provide for that coordination? Are there existing interface agreements, documented APIs, or data exchange specifications that will be made available?

[Coordination points and points of contact for existing VAERS support contractors will be identified after award; specific interface agreements are not releasable at this time.](#)

218. The solicitation describes the current environment as including Azure SQL Managed Instance, virtual machines, ColdFusion, Tomcat, Node.js, .NET, Akamai, Blob Storage, Key Vault, and Logic Apps, with the reporting interface supporting writable PDF, CXone, and eFax. Can CDC describe the expected integration points for the new front end, and identify which components the new application must integrate with versus replace?

[See PWS Subsection D for the current architecture summary \(Azure SQL Managed Instance, VMs, ColdFusion, Tomcat, Node.js, .NET, Akamai, Blob Storage, Key Vault, Logic Apps\). Scope is limited to developing the new reporting application and integrating with existing VAERS services; wholesale replacement is not required. Additional documentation will be furnished at kickoff.](#)

219. Is the CDC-managed Azure environment already provisioned for this effort, or does standing up the development, test, and deployment environment fall within the period of performance? What is the expected lead time between PIV issuance and developer access to the environment?

Standing up the specific development/test/deployment environment configuration falls within the period of performance. Expected lead time is not specified; offerors should build reasonable ramp-up time into their schedules.

220. Task 1.10 requires reducing "the submission abandonment rate" by at least 30 percent without identifying a submission channel. Does this requirement apply only to the modernized web reporting form, or to all VAERS submission methods including writable PDF, eFax, and CXone? If it applies across channels, how does the Government define abandonment for non-web submission methods?

See answer to question 44.

221. The 30 percent reduction is measured against a baseline "established at project kickoff." How will that baseline be measured, specifically: what population constitutes the denominator, what instrumentation will be used, and over what observation window? Will the Government provide the baseline value, or is establishing the baseline a Contractor responsibility?

See answer to question 44.

222. PRS#4 requires the Contractor to measure and report against Task 1.10 targets "using approved methodology." Who approves the measurement methodology, by when must it be approved, and is the methodology a separate deliverable? It does not appear in the PWS Section 8 deliverables schedule.

See answer to question 44. It is not a separately scheduled deliverable in Section 8 but should be documented as part of Task 1.10 measurement reporting.

223. PWS Section 9 lists data element definitions, business rules, integration requirements, and ATD/Azure security requirements as the reference materials provided at kickoff. Will CDC also provide existing user research, usability testing findings, help desk or FAQ inquiry data, and web analytics for the current VAERS reporting experience? These would materially inform the customer-centric design, FAQ Data Report, and performance measurement requirements.

Available materials will be furnished at kickoff to the extent they exist and are releasable.

224. Is the current VAERS reporting interface instrumented for web analytics or user behavior measurement? If so, what tooling is in place, and will the Contractor have access to it at kickoff?

Any existing instrumentation and access details will be addressed at kickoff.

225. PRS#5 establishes a data completeness standard of at least 90 percent for required and critical VAERS data elements. Is a current completeness baseline available, and will the Government define the specific data elements considered "required and critical" for measurement purposes?

The Government will define required and critical data elements at kickoff; a current baseline, if available, will be furnished at that time.

226. Has OMB PRA clearance already been obtained for the modified VAERS reporting form and for the two new customer satisfaction survey instruments, or is clearance required within this period of performance? Page 34 states that the Government will determine PRA applicability; please confirm the current status of that determination.

[See answer to question 5.](#)

227. If PRA clearance is required, who owns preparation and submission of the clearance package, CDC or the Contractor? Given that HHSAR 352.211-3(b) directs the Contractor to allow at least 120 days for OMB clearance, and that the survey instruments are due with Beta Release at DOA + 90 days, how should offerors reflect this in their proposed schedule?

[See answer to question 5.](#)

228. CDCH.04 identifies key personnel as essential to contract performance and includes a "Personnel Title" heading with no positions listed. Does the Government have required key personnel positions or labor categories, or may offerors define their own? Will CDCH.04 be populated at award?

[Offerors may propose their own Key Personnel positions consistent with their technical and management approach. The Government did not identify any specific positions as key personnel positions. CDCH.04 will be populated based on the offeror's proposal.](#)

229. Please confirm that key personnel resumes, at 2 pages per resume, are excluded from the 15-page Technical Proposal limitation.

[See answer to Question 3.](#)

230. Task 1.8 requires a low-code/configurable interface, while Standard-11 requires that all software first produced under the contract be open-source software with data rights under FAR 52.227-14. Would use of a commercial low-code platform satisfy Task 1.8, and how does CDC reconcile a proprietary platform with the Federal Source Code Policy obligation? Standard-11 permits exceptions "unless otherwise specified in writing by the contract officer" - would such a written exception be issued for this requirement?

[It is possible that a commercial low-code platform would satisfy the task. It is still subject to FEDRAMP and related security requirements.](#)

231. Is there a CDC-approved list of low-code platforms, or an existing CDC enterprise license (for example, within EDAV or ICDP) that offerors should assume is available for this effort?

[See answer to Question 109.](#)

232. In line with the RFO companion, will the government provide an affordability statement to aid offerors in providing innovative solutions?

[See answer to question 57.](#)

233. Section F directs that each IT supply, including licenses and cloud services, be a separately priced line item, while PWS Section 5 states that CDC will provide access to the CDC-managed Azure environment. Are Azure compute, storage, and licensing costs borne by CDC's existing environment, or must offerors price them as separately priced IT supply line items?

[See answer to Question 106.](#)

234. CLINs 0002 and 0003 are each shown as \$31,500.00 estimate and \$31,500.00 not-to-exceed for onboarding and meeting travel, while PWS Section 7 states that no travel other than that necessary for employee onboarding is anticipated. Should offerors price these CLINs at the stated not-to-exceed amounts, or propose amounts based on their own estimates?

Per Section 7: No travel, other than that necessary for employee onboarding, is anticipated for this contract. The Contractor shall perform work remotely and coordinate with CDC via virtual meetings. *If the Government requests travel (e.g., for in-person meetings at CDC Atlanta), travel must be approved in advance by the COR and shall be reimbursed in accordance with applicable Federal Travel Regulation (FTR) requirements and the travel terms of the contract.*

235. Can the government kindly confirm that this requirement is a follow-up on the GSA Market Research titled "Modernized Application for VAERS Reporting," which was released in January 2026?

Yes, this is the same general requirement; however, there have been some updates.

236. Can the government kindly identify the incumbent contractor who previously performed the work described within this RFQ?

There is no incumbent contractor.

237. Can the government kindly confirm whether the incumbent contractor is eligible to compete for this award?

There is no incumbent contractor.

238. The PWS establishes deliverables at 10, 21, 30, 45, and 90 calendar days after date of award, while CDC will furnish contractor laptops, PIV credentials, and access to the CDC-managed Azure environment. If required Government-furnished equipment, credentials, system access, security approvals, or development environment access are not available at the date of award, will affected milestone dates be adjusted accordingly?

Yes.

239. If an Offeror submits an Artificial Intelligence Compliance and Risk Management Plan with its proposal and that plan is found acceptable during evaluation, will award constitute approval for the AI uses described in the plan, or is a separate post-award approval required before those AI capabilities may be used?

An AI Plan found acceptable during evaluation is expected to carry forward at award. Use beyond the scope described in the accepted plan requires a revised submission and approval.

240. May an approved AI Compliance and Risk Management Plan authorize recurring generative-AI-assisted software development activities throughout the SDLC, including source-code generation, code review, refactoring, test generation, documentation generation, accessibility remediation, security remediation, infrastructure-as-code assistance, and development planning, provided such use remains within the approved data-handling, hosting, model, and human-review controls?

Yes, provided such use remains within the data-handling, hosting, model, and human-review controls described in the approved AI Compliance and Risk Management Plan.

241. For purposes of CDCH.10, does a model-version change within an already approved provider or managed AI service require submission and approval of a revised AI plan, or may an Offeror identify and obtain approval for a defined model family or managed AI service?

Offerors may identify and seek approval for a defined model family or managed AI service; materially different models or providers would require a revised submission.

242. Does CDC permit contractor use of commercially hosted generative AI coding tools when Government non-public information, CUI, PII, PHI, credentials, secrets, production data, and other protected Government information are excluded from the AI system, subject to an approved AI Compliance and Risk Management Plan?

Yes, subject to an approved AI Compliance and Risk Management Plan and confirmation that no Government non-public information, CUI, PII, PHI, credentials, or production data are processed by the external tool.

243. Will software development be required to occur exclusively on CDC-furnished equipment? If so, may contractor personnel access and authenticate to approved commercial generative AI development services from CDC-furnished laptops, including browser-based AI services, IDE-integrated coding assistants, or command-line AI coding tools, when such use is covered by an approved AI Compliance and Risk Management Plan? Please also clarify whether CDC-managed devices will permit installation or use of the necessary IDE extensions, command-line tools, package managers, source-control clients, container tooling, and outbound network access required for those approved services.

Development is expected to occur on CDC-furnished equipment where required by security policy. Use of approved external AI tools from CDC-furnished equipment, and associated tooling/network access, is subject to CDC's standard device management policies and will be coordinated at kickoff.

244. If commercial generative AI services are approved for software-development use but cannot be accessed directly from CDC-furnished equipment, does CDC provide an approved alternative, such as Azure OpenAI or another Government-authorized AI development environment, that contractors may use for AI-assisted software engineering?

CDC's Azure OpenAI services within the EDAV environment are available as an alternative as well as other HHS approved models.

245. Does use of generative AI solely for development of the pre-award proposal prototype constitute AI use in contract performance requiring Government approval, or do the CDCH.10 requirements apply to AI use during contract performance following award?

CDCH.10 approval requirements apply to AI use during contract performance following award, not to an offeror's internal tools used solely to develop the pre-award proposal prototype.

246. For the Tab 2-2 proposal prototype, may Offerors use synthetic/non-sensitive test data and mocked interfaces for existing VAERS integrations that will not be available until after award, provided the prototype demonstrates the proposed workflows, branching logic, functionality, architecture, and integration approach?

Yes. See answer to question 28.

247. For Tab 2-2, is CDC seeking a functional proof of concept demonstrating representative end-to-end workflows, or is the expectation that the proposal prototype implement substantially all functional requirements identified in Tasks 1 and 2?

Demonstration of functional requirements identified in the table and any other improvements. See answer to question 28.

248. Is the Day-45 Alpha expected to integrate with live or test instances of existing VAERS systems, or may external VAERS interfaces be mocked or stubbed until CDC provides the applicable integration specifications, connectivity, credentials, and approvals?

External VAERS interfaces may be mocked or stubbed until CDC provides applicable integration specifications, connectivity, credentials, and approvals.

249. Where delivery depends upon interfaces, access, technical information, approvals, or actions from incumbent VAERS contractors, CDC hosting teams, or other Government support organizations, how will schedule impacts resulting from delayed external dependencies be handled?

Milestone dates will be adjusted to the extent delays are attributable to Government or Government-designated dependencies.

250. Because the application is to be developed and deployed within the CDC-managed FedRAMP-authorized Azure environment, please confirm whether the contractor is expected to obtain a separate FedRAMP authorization or procure a separate 3PAO assessment for the application, or whether the contractor is responsible for application-level controls, documentation, remediation, and support of CDC's ATD/ATO process within the existing authorized environment.

See questions 9 and 251.

251. Are independent penetration testing, 3PAO assessment, or other third-party security assessment costs expected to be included in the contractor's Firm Fixed Price, or will applicable CDC/Government security organizations perform those assessments against the application within the CDC-managed environment?

For the CDC managed cloud environment these controls are inherited. CDC will work with the contractor to obtain the application-level artifacts such as scans and other security controls to support CDC authorizations.

252. Does CDC require any contractor-held HIPAA certification, third-party HIPAA certification, or similar attestation as a condition of award or performance, or is HIPAA compliance demonstrated through implementation and documentation of the applicable privacy and security safeguards within the CDC-managed environment?

No specific third-party HIPAA certification is required; compliance is demonstrated through implementation and documentation of applicable safeguards.

253. The PWS specifies a nine-month period of performance while the Production-Ready Final Release is due at DOA + 150 days. Please clarify the expected contractor responsibilities and level of effort for the remainder of the period of performance following final release, including O&M, production support, defect remediation, enhancement work, and transition activities.

See answer to Question 13.

254. Are there Government-designated mandatory Key Personnel positions or minimum staffing levels for this requirement, or may the Offeror determine the appropriate Key Personnel, labor categories, staffing levels, and staffing mix necessary to accomplish the PWS?

[See answer to Question 33.](#)

255. For the Similar Experience factor, may the Offeror rely on relevant experience of proposed Key Personnel and/or subcontractors, including experience performed while employed by other organizations, or is the evaluation limited to projects previously performed by the Offeror as a corporate entity?

[See answer to Question 33.](#)

- Page 33 identifies an Incident Response Plan as due "as part of Proposal." Please confirm whether the Incident Response Plan is required with the quotation and, if so, whether it is included within the 15-page Technical Proposal limit or submitted as a separate attachment outside the page limit.

[See answer to Question 3.](#)

256. Please confirm the Section 508 documentation required at proposal submission for a custom solution that has not yet been developed. May the Offeror submit the applicable HHS Accessibility Conformance Checklist describing the proposed approach and planned conformance, with the final ACR/VPAT delivered at Beta as identified in the PWS?

[Yes, required. It is submitted as a separate attachment and excluded from the 15-page limit; it is evaluated on a pass/fail basis similar to the DMP and AI Plan.](#)

257. To the extent implementation or testing of the redesigned reporting workflow or customer satisfaction surveys requires PRA/OMB clearance, will CDC be responsible for obtaining the applicable clearance, and will Government-caused clearance delays result in adjustment of affected implementation or acceptance milestones?

[See answer to question 5.](#)

258. Please confirm the following: Section 4, Task 3.2 3.2 The Contractor shall ensure the application is developed and deployed within the CDC-managed FedRAMP-authorized Azure environment approved for use by CDC, consistent with Government security requirements.

[Confirmed.](#)

259. FAR 52.219-4 (Notice of Price Evaluation Preference for HUBZone Small Business Concerns) is incorporated in Section D, while the SF-18 indicates a small business set-aside. Will the HUBZone price evaluation preference be applied in the evaluation of quotes under this RFQ?

[FAR 52.219-4 is removed from the solicitation.](#)

260. Section B states a period of performance of 28 September 2026 through 27 June 2027, while the Section 8 deliverable table states that Testing and Final Release (Production-Ready) is due "By end of PoP (DOA + 150 days)". Please confirm the intended due date for Final Release and the intended activity for the remainder of the period of performance.

[See answer to Question 13.](#)

261. Will the modernized application submit reports into the existing CDC VAERS system of record, or into FDA's Adverse Event Monitoring System (AEMS)? Please identify the receiving interface, its owner, and whether an interface specification will be furnished at kickoff.

[The application reports into the existing CDC VAERS system of record. Interface ownership and specifications will be identified at kickoff.](#)

262. Regarding the Authority to Develop (ATD): what specific artifacts must the Contractor submit, which official issues it, and what has been the historical median time from documentation submission to ATD issuance for systems in the VAERS boundary? We are familiar with ATT that allows development to start, and ATT in process while resources are provisioned for Dev/Test.

[CDC, in coordination with OCIO, issues the ATD.](#)

263. Will an Authority to Operate (ATO) be required for production deployment within this period of performance, and if so, is the Contractor responsible for the full SA&A package or for supporting a CDC-led authorization? Or, would the government consider a Modification of the existing VAERS ATO to add the new functionality in order to reduce the timeframe a full SA&A package may introduce?

[See answer to Question 9.](#)

264. Task 1.9 requires VAERS-compatible data capture and transmission. Will CDC furnish the VAERS data element definitions, business rules and integration requirements at kickoff, as indicated in Section 9, and can a draft data dictionary be made available during the quotation period?

[Yes, at kick off data element definitions, business rules and integration requirements will be provided. Offerors should base the prototype on the publicly available current VAERS form and general PWS requirements. The current form can be found at \[VAERS - Download / Upload a Writable PDF Form\]\(#\) and the VAERS Data Use Guide found at \[VAERS - Data Sets\]\(#\) provides field names and form box alignment \(derived fields are related to this RFP\).](#)

265. Task 1.10 requires reducing submission abandonment by at least 30% against a baseline established at project kickoff. Does CDC currently measure abandonment on the existing form, and will baseline data be furnished?

See answer to Question 44.

266. Task 1.8 requires a low-code/configurable interface. Does CDC have an existing preferred content management or configuration platform within the CDC Azure environment that the Contractor should use, or is the mechanism at the Contractor's discretion?

See answer to Question 109.

267. For Tab 2-2, please confirm acceptable prototype hosting: may the prototype be hosted on Contractor-controlled infrastructure outside the CDC boundary, and are there any restrictions on the technologies or data used in the prototype?

Yes, prototypes may be on Contractor controlled infrastructure, and no restrictions as long as the prototype tech stack uses the same tech stack available to CDC and are part of FedRAMP approved infrastructure.

268. Please confirm whether the existing VAERS system operator will be made available for technical exchange during performance, and how integration dependencies between the two efforts will be coordinated.

Coordination points and points of contact for existing VAERS support contractors will be identified after award.

269. Will CDC issue an amendment with a Q&A response, and by what date, given the 4 September closing?

Yes.

270. The current online form provides item instructions in English and Spanish. Is Spanish parity required in the modernized application, and are any additional languages expected?

English and Spanish are the languages expected.

271. Does the current online form distinguish public from healthcare-provider submitters in any way, or is the branching required by PWS 1.6 entirely new to this acquisition?

No, this is a new feature.

272. Does the Government anticipate any period of performance beyond the nine-month base period, whether through options or a follow-on requirement?

There are no current plans to extend.

273. Section 3 references coordination with other existing contracts supporting VAERS. Can you identify those contracts or contract holders so we can appropriately account for integration dependencies in our technical approach?

Coordination points and points of contact for existing VAERS support contractors will be identified after award.

274. Can the deadline be extended?

The deadline for quotations is extended to 9/8/2026 10:00 AM ET.